RWANDA NATIONAL
ASSOCIATION OF DEAF WOMEN
(RNADW "UMUCYO")

No Sign Language No Rights

FULFILLING RWANDA'S SRHR PROMISE FOR DEAF GIRLS

POLICY BRIEF

KIGALI, RWANDA
SEPTEMBER 2025

HERE IS THE POLICY BRIEF

IN NUMBERS

20
In-depth interviews with Deaf AGYW.

12,000
Deaf adolescent girls and young women are failed by a sexual and reproductive health and rights (SRHR) system.

20
In-depth interviews with parents of the deaf.

66,272
Individuals in Rwanda live with hearing difficulties.

20
In-depth interviews with community health workers.

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WITHOUT SIGN LANGUAGE, THERE ARE NO RIGHTS.

1. EXECUTIVE SUMMARY

Deaf adolescent girls and young women (Deaf AGYW) in Rwanda are systematically excluded from accessing sexual and reproductive health and rights (SRHR) due to intersecting barriers of communication, gender bias, and ableism.

This denial of autonomy and care violates their rights, compromises public health, and undermines Rwanda's commitments to equality. Consultations revealed that AGYW are invisible in policy and face stigma and pity instead of support. This brief calls for urgent, multi-level action to integrate Rwandan Sign Language (RSL) into SRHR services and information, challenge discriminatory norms, and implement intersectional policies. Investing in inclusion is a moral, legal, and public health imperative to ensure no one is left behind.

2. INTRODUCTION

This policy brief highlights critical gaps in sexual and reproductive health and rights (SRHR) for Deaf adolescents and young people in Rwanda, focusing on the role of discriminatory social and gender norms.

Despite national commitments to inclusion, Deaf AGYW youth remain systematically overlooked in SRHR programming and policy design, their needs masked within broad youth or disability frameworks. Evidence from recent consultations reveals that harmful norms, including ableism, audism, and gender bias, are pervasive, socially enforced community expectations across families, schools, and health institutions. This brief examines how these norms intersect with structural barriers like communication exclusion to limit SRHR access. It argues for deliberate, multi-level interventions that centre Deaf voices and target norms change as a critical strategy for achieving SRHR for all.

3. BACKGROUND

An estimated **66,272** people in Rwanda live with hearing difficulties, excluding children under five (National Institute of Statistics of Rwanda, 2022).

Extrapolating from this demographic data, it is estimated that approximately **12,000** Deaf adolescent girls and young women are currently failed by a sexual and reproductive health and rights (SRHR) system that is not designed to accommodate their needs. Despite Rwanda's strong policy commitments to gender equality (Revised National Gender Policy, 2021) and disability inclusion (National Policy on Disability, 2021), a significant implementation gap persists, allowing this population to fall through the cracks.

This systemic exclusion carries severe immediate and long-term consequences, including elevated risks of unplanned pregnancy, sexually transmitted infections, and sexual violence. Furthermore, it results in the forfeiture of educational and economic potential and constitutes a perpetual violation of fundamental human rights[1]

Rwanda's policy framework is robust on paper. The Health Sector Strategic Plan V prioritises SRHR and adolescent health, while the National Policy on Disability and Inclusion (2021) commits to removing barriers across sectors, aligning with CRPD obligations. Progress includes the development of a Rwandan Sign Language Dictionary (2023) and sign language interpretation on public television.

However, for Deaf adolescents, communication remains the central bottleneck. Access to RSL in health settings is inconsistent, especially outside urban centres. While policies endorse disability-responsive service delivery, the reality is an intersection of gendered stigma, ableism, economic dependency, and geographic inequity that depresses demand and limits care quality. Closing this gap requires embedding RSL competencies into SRHR delivery, financing interpreter services, and pairing health services with socio-economic support.

1 Africa Press Rwanda, 2022

Key Concepts: Understanding Ableism and Audism

Ableism: Discrimination and social prejudice against people with disabilities based on the belief that typical abilities are superior.

Audism: A form of ableism that prioritises spoken speech over sign language and treats Deaf people as inferior to hearing people.

WHY EXISTING STRATEGIES ARE FAILING DEAF AGYW

Rwanda's current approach to SRHR and disability inclusion is siloed, leading to three inadequate pathways for addressing the needs of Deaf AGYW:

- **Mainstreaming in Gender Policies:** SRHR programs targeting "all women and girls" lack RSL interpreters and accessible materials, rendering Deaf AGYW invisible. This assumes inclusion without providing the tools for it.
- **Generic Disability Inclusion:** Disability policies often focus on physical access (ramps) or broader categories, neglecting the specific linguistic and communication barriers faced by the Deaf community. This approach does not mandate sign language proficiency.
- **NGO-Led Projects:** Interventions are often short-term, project-based, and dependent on external funding (for example, the noted 2023 RSL dictionary). This creates patchy, unsustainable coverage and fails to institutionalise change within public systems.

These pathways are not mutually exclusive but are currently operating in parallel.

The result is a systemic failure to address the intersectional nature of the barriers Deaf AGYW face. A new, integrated approach is required.

HOW SOCIAL NORMS REINFORCE EXCLUSION FOR DEAF AGYW

Social norms, unwritten rules governing behaviour, powerfully influence SRHR outcomes, especially during adolescence. For Deaf AGYW, harmful norms are experienced as silence, stigma, and surveillance, compounded by communication barriers. These norms are reinforced at multiple levels:

- **Families:** Decisions about health are made for them, not with them; SRHR topics are taboo, limiting knowledge and autonomy.
- **Peers:** Communication barriers exclude them from informal learning, leading to isolation and misinformation.
- **Schools:** A lack of teachers trained in RSL and inclusive sexuality education sidelines Deaf students, reinforcing irrelevance.
- **Service Providers:** Health workers' lack of RSL skills leads to miscommunication; speaking to parents instead of patients undermines consent and trust.
- **Systems:** The absence of interpreters, accessible protocols, and disaggregated data institutionalises exclusion.
- **Policies:** Siloed gender and disability frameworks mean Deaf AGYW belong to both yet are served by neither, perpetuating their invisibility.

The cumulative impact of these reinforcing norms across different settings is a self-perpetuating cycle of exclusion.

This cycle, depicted in Figure 1 below, illustrates how negative perceptions and institutional barriers directly lead to poor SRHR knowledge and outcomes for Deaf AGYW, which in turn feed back into and reinforce the very norms that started it.

This conceptual model provides a framework for understanding the systemic nature of the problem.

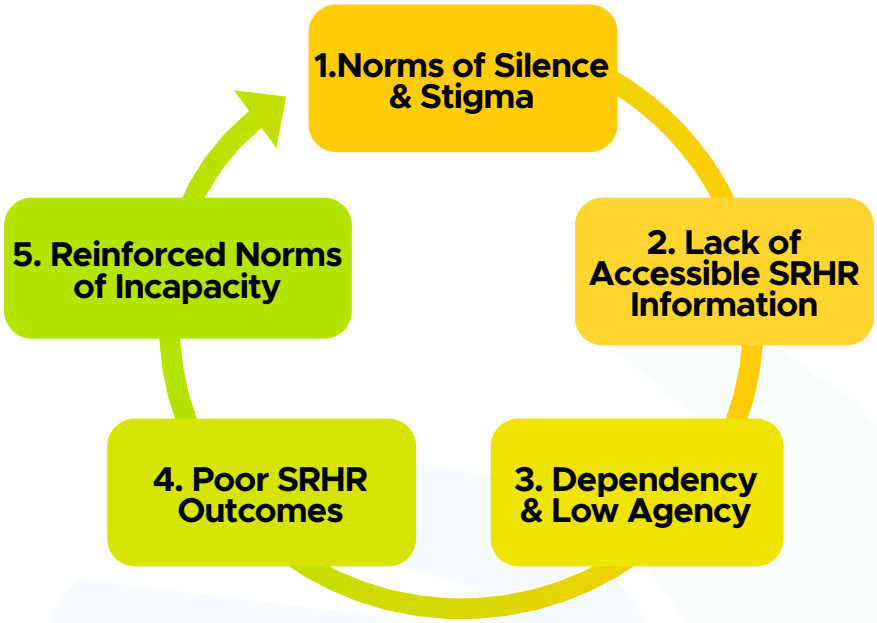

Figure 1: The Cycle of Exclusion Facing Deaf AGYW in SRHR

The following findings from our field research offer concrete evidence of this cycle, as shown in Figure 1 in action, detailing how these barriers manifest in the daily lives of Deaf AGYW in Rwanda.

FINDINGS

Data from in-depth interviews with **20** Deaf AGYW, **20** parents, and **20** community health workers in Huye and Nyarugenge districts reveal consistent patterns of exclusion. The table below summarises the key barriers and their impacts.

Table 1: Summary of Key Barriers Faced by Deaf AGYW in Accessing SRHR

Barrier Category	How It Manifests	Impact on Deaf AGYW
Intersectional Discrimination	Overlooked in gender/ disability programs; spoken about, not to; denied privacy.	Statistical and programmatic invisibility; loss of autonomy.
Ableism in Healthcare	Viewed as "too complicated to serve"; assumptions of incapacity.	Missed preventative care; skipped counselling; uninformed choices.
Audism in Communication	Lack of RSL proficiency among providers leads to the use of improvised gestures or family interpreters.	Breaches of confidentiality, weakened informed consent, and misinformation.
Stigma & Stereotyping	Perceived as either asexual/ childlike or lacking morality.	Erasure of sexual agency; delayed SRHR knowledge; fear of seeking care.
Economic Exclusion	High unemployment and dependency limit funds for transport and services.	SRHR needs are postponed or ignored; lack of control over care-seeking.
Invisibility in Policy	Siloed policies; lack of Deaf-specific indicators or mandates for RSL access.	Policies fail to translate into real-world accommodations (interpreters, materials).
Dependency Reinforcement	Adults speak on their behalf in healthcare settings.	Undermines the right to confidential, autonomous healthcare.

In Huye District, one participant recalled:
"When I was at the health centre, they ignored me and spoke to my mother. I could see they were talking, but I did not know what they were saying."

Deaf girls reported that health workers communicated with their parents rather than them. One said,
"They didn't ask me. They asked my mother. I was there, but it felt like I wasn't."

As one Kabutare parent put it:
"They think if they do something for women, it will include our daughters. But our daughters are forgotten."

In Nyarugenge, one girl explained:
"Some people think we cannot have boyfriends or get pregnant. They treat us like children."

This exclusion is confirmed by national evidence, including reports from the National Council of Persons with Disabilities (2023) and UNFPA Global Guidance on Intersectional SRHR Programming (2022), which stresses that mainstreaming alone is insufficient without targeted, intersectional interventions.

4. RECOMMENDATIONS

Table 2: Multi-Level Strategies for Inclusive SRHR for Deaf AGYW

Stakeholder	Recommendation	Key Actions
Government of Rwanda	Formalise Rwandan Sign Language (RSL) as an official language and ensure its implementation in public services	* Pass legislation granting RSL official language status. * Expedite the completion and official adoption of the RSL dictionary. * Fund the development of a national corps of trained, certified RSL interpreters. * Mandate the availability of free interpreter services in all public health centres.
Government of Rwanda	Integrate RSL and disability inclusion into core health training and budgets.	* Mandate certified RSL training for SRHR providers. * Establish budget lines for interpreters and accessible materials. * Include Deaf-specific indicators in health monitoring systems.
Government of Rwanda	Enforce intersectional policy implementation.	* Conduct joint audits (GMO, NCPD, Min. of Health) on inclusion. * Revise policies to explicitly mandate RSL access.
Civil Society & DPOs	Lead community-level norm change campaigns.	* Co-design anti-stigma initiatives with Deaf women leaders. * Disseminate SRHR information in RSL (videos).
Development Partners & NGOs	Fund integrated programming.	* Finance programs combining SRHR education with vocational training. * Support research on Deaf AGYW's SRHR outcomes.
All Stakeholders	Centre Deaf leadership and participation.	* Ensure meaningful consultation with Deaf Adolescent Girls and Young Women's organisations. * Employ Deaf peer educators and community health workers.

5. CONCLUSION

Deaf AGYW face intersecting barriers to SRHR that cannot be solved by addressing gender or disability in isolation. Policymakers, donors, and service providers must adopt the intersectional strategies outlined above to address communication barriers, challenge stigma, enable economic empowerment, and ensure accountable policy implementation.

Addressing this crisis is a legal, moral, and public health imperative. By acting now, Rwanda can transform its strong policy commitments into tangible change, ensuring Deaf girls are seen, heard, and empowered to make decisions about their own bodies and futures.

This will not only uphold their fundamental rights but also contribute to a healthier, more equitable, and truly inclusive society for all.

5.1. Recommended Sources

- UNDP Gender and Disability Analysis for Rwanda, 2024.
- National Council of Persons with Disabilities (NCPD) Annual Report, 2023.
- Rwanda Governance Board, Disability Inclusion Analysis, 2023.
- Handicap International, Gendered Experiences Report, 2022.
- UNFPA Global Guidance on Intersectional SRHR Programming, 2022.
- Fifth Rwanda Population and Housing Census, 2022
- The Indepth Interview Guide can be accessed here: Consent Form and Interview Guide

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